



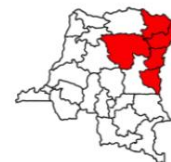
EBOLA PRESIDENTIAL FORCE TASK 17

Ministry of Public Health, Hygiene and Social Welfare

National Institute of Public Health

Public Health Emergency Operations Center

MVE17 Incident Management System



Situation Report of the 17th Ebola Virus Disease Outbreak / DRC

SitRep N°113/MVEBDB/04/09/2026

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HIGHLIGHTS (last 24 hours)

As of September 4, 2026, 86 new confirmed cases of Ebola Virus Disease (EVD), including 30 community deaths, were recorded in the provinces of Ituri (53 cases), North Kivu (31 cases), and Haut-Uélé (2). However, 9 deaths intra-CTE were recorded.

A new health zone has been affected in the last 24 hours in North Kivu, namely that of Kayna.

ACCUMULATIVE CASE 6,522	ACCUMULATIVE DEATH CONFIRMED 3,134	LETHALITY 48.1%	PATIENTS IN ISOLATION/CTE 817	ACCUMULATIVE HEALED 1,516	TRACKING RATES CONTACTS (Of the Day) 86.0%
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Affected Provinces and Affected Health Zones



6 Provinces Affected

Haut-Uélé, Ituri, North Kivu,
South Kivu, Tshopo & Bas Uélé



61 out of 151
health zones affected
(40.4%)



Summary of key points from the last 24 hours

There has been sustained reporting with **86 new confirmed cases** of

Ebola Virus Disease (EVD) has resulted in **30 community deaths** in the last 24 hours, with a new health zone affected in North Kivu (**Kayna**). These cases are distributed across the provinces of **Ituri (53 cases) and North Kivu.**

(31) and Haut Uélé (2). However, **9 intra-CTE deaths** were also recorded, including 7 in Ituri, 1 in North Kivu and 1 in Haut Uélé, bringing **the number of deaths for the day to 39.**

With bravery, **21 patients** were declared cured after obtaining two negative results for Ebola Virus Disease (19 in Ituri and 2 in North Kivu).

As of September 4, 2026, the DRC has recorded **6,522 confirmed cases, including 3,134 deaths.** representing a case fatality rate of **48.1 %**. The epicenter remains Ituri province with **80.9%** of confirmed cases recorded in the country and concentrates **61.6%** of new confirmed cases reported in the last 24 hours while North Kivu comes in second position (36.0% of new cases).

The new health zone (Kayna) assigned to North Kivu brings the total number of affected **health zones to 61 out of 151 (40.3%)** across the 6 affected provinces. Ituri has 28 out of 36 affected health zones, followed by North Kivu (16/34), Haut-Uélé (6/13), Tshopo (7/23), South Kivu (1/34), and Bas-Uélé (3/11).

Table 1. Distribution of confirmed cases and deaths by affected province, as of September 4, 2026.

Province	New confirmed cases (24h)	Confirmed cases	Deaths (confirmed)	Lethality	Affected health zones
Ituri	53	5277	2,378	45.1%	28/36 (77.8%)
North Kivu	31	969	638	65.8%	16/34 (47.0%)
Haut-Uélé	2	247	105	42.5%	6/13 (46.1%)
Tshopo	0	22	9	40.9%	7/23 (30.4%)
South Kivu	0	3	1	33.3%	1/34 (2.9%)
Bas Uélé	0	4	3	75.0%	3/11 (27.3%)
Total	86	6,522	3,134	48.1%	61/151 (40.4%)

Table 2. Distribution of confirmed cases and deaths by province and health zone, as of September 4, 2026.

Province / Health Zone	Cumulative number			Current situation (24 hours)			
	Case (n)	Deaths (n)	Lethality	New Case	Community deaths (Swab+)	Confirmed deaths within the CTE	Total confirmed deaths
Ituri	5277	2378	45.1%	53	16	7	23
Adja	11	1	9.1%				0
Aru	8	6	75.0%				0
Ariwara	8	2	25.0%				0
Aungba	12	5	41.7%				0
Bamboo	171	36	21.1%	1	0		0
Boga	2	2	100.0%				0
Bunia	1471	456	31.0%	16	7		7
Damascus	30	9	30.0%				0
Drodro	15	7	46.7%	1	0		0
Fataki	77	30	39.0%				0
Gethy	6	2	33.3%				0
Kambala	2	0	0.0%				0
Kilo	32	12	37.5%				0
Komanda	104	70	67.3%	6	4		4
Lita	234	121	51.7%	3	0		0
Logo	13	5	38.5%	1	0		0
Lolwa	25	7	28.0%	3	2		2
Mahagi	4	1	25.0%				0
Mambasa	17	7	41.2%				0
Mandima	38	21	55.3%	2	0		0
Mangala	287	140	48.8%	9	2		2
Mongbwalu	624	288	46.2%	1	0		0
Nia-Nia	221	113	51.1%				0
Nizi	688	267	38.8%	5	0		0
Nyankunde	124	35	28.2%				0
Rimba	11	4	36.4%				0

Rwampara	982	345	35.1%	5	1		1
Tchomia	60	25	41.7%				0
To ventilate	N / A	361	N / A	N / A	N / A	7	7
North Kivu	969	638	65.8%	31	14	1	15
Blessed	157	115	73.2%	1	0		0
Good	2	1	50.0%	1	0		0
Butembo	197	138	70.1%	4	2		2
Goma	1	0	0.0%				0
Kalunguta	17	8	47.1%				0
Katwa	440	286	65.0%	13	7	1	8
Kayna	1	0	0.0%	1	0		
Kyondo	17	12	70.6%	1	0		0
Lubero	5	4	80.0%	1	1		1
Mabalako	7	5	71.4%				0
Manguredjipa	3	2	66.7%	1	1		1
Masereka	13	5	38.5%				0
Musienene	92	50	54.3%	7	2		2
Mutwanga	2	2	100.0%				0
Oicha	8	6	75.0%				0
Vuhovi	7	4	57.1%	1	1		1
Haut-Uélé	247	105	42.5%	2	0	1	1
Boma Mangbetu	29	14	48.3%				0
Gombari	3	1	33.3%				0
Isiro	90	36	40.0%	1	0		0
Pawa	40	22	55.0%	1	0		0
Rungu	2	1	50.0%				0
Wamba	83	31	37.3%			1	1
Tshopo	22	9	40.9%	0	0	0	0
Bafwasende	2	0	0.0%				0
Kabondo	4	1	25.0%				0
Lubunga	1	0	0.0%				0
Makiso-Kisangani	10	5	50.0%				0
Mangobo	3	2	66.7%				0
Tshopo	1	0	0.0%				0
Wanie-Rukula	1	1	100.0%				0
South Kivu	3	1	33.3%	0	0	0	0
Miti-Murhesa	3	1	33.3%				0
Bas Uélé	4	3	75.0%	0	0	0	0
Buta	1	1	100.0%				0
Viadana	1	0	0.0%				0
Ganga	2	2	100.0%				0
Total	6522	3134	48.1%	86	30	9	39

*These are the confirmed deaths that occurred in the various CTEs in the province and are being broken down to be classified by health zone.

NA: Not Applicable.

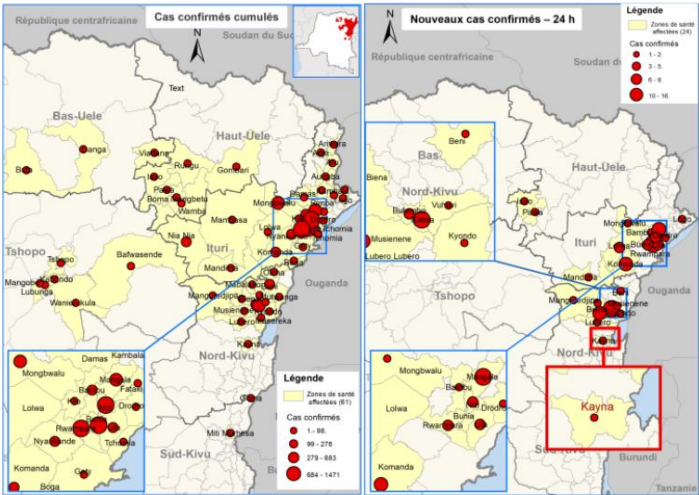
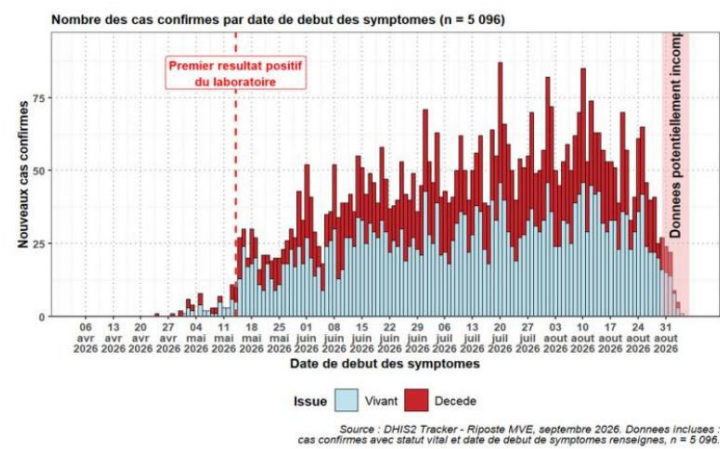
** These deaths were recorded in the various CTEs of the Ituri province and are currently being assessed.

Table 3. Status of alerts notified by province, as of September 4, 2026

DPS	Alerts Received		Total alerts	Alerts verified and validated (suspected case)		Alerts verified and invalidated (no suspected cases)		Number of suspects investigated	Number of suspects investigated and transferred to CT/CTE (of the day)
	Living	Deceased		Living	Deceased	Living	Deceased		
Haut-Uélé	18	6	24	18	5	0	0	23	11
Bas Uélé	28	2	30	4	1	24	1	5	0
Ituri	745	74	819	142	64	429	6	206	121
North Kivu	1104	54	1158	100	54	994	0	154	83
South Kivu	8	0	8	5	0	3	0	5	5
Tshopo	104	1	105	5	1	99	0	6	2
Total General	2007	137	2,144	274	125	1549	7	399	222

Figure 1. Evolution of cases by date of symptom onset over the last 21 days

Figure 2. Spatial distribution of confirmed cases (24h and cumulative) by health zone.



1. COORDINATION

1.1. Main Actions

- Monitoring and support of response actions in all affected provinces.

2. RESPONSE ACTIONS BY PILLAR

1.1.1. Epidemiological surveillance

1.1.2. Main Actions

For the day of September 4, 2026, 2,144 alerts were recorded in the 6 Of the affected provinces, 1,955 (91.2%) were verified. Thus, all 399 alerts validated as suspected cases were investigated (100%) and 222 of them were transferred to CT/CTE (Table 3).

Regarding contact tracing, the daily proportion stands at 86.0% (21,909 seen out of 25,500 to be followed) slightly down compared to the previous day (88.5%), approximately above the threshold of 85%; 89.8% in Ituri (12,432/13,839), 83.3% in North Kivu (7,910/9,491), 73.9% in Tshopo (246/333), 72.1% in Haut Uélé (1,215/1,685) and 69.7% in Bas Uélé (106/152).

1.1.3. Challenges

Insufficient teams dedicated to surveillance activities, and more specifically to contact tracing.

1.2. Update on checkpoint and point of entry (PoC/PoE) activities**1.2.1. Main actions**

Nine (9) alerts were notified to the various PoE/PoCs as of September 4, 2026. In North Kivu, two alerts were recorded: a deceased body intercepted at PoC PK5 (PAida Health Center, Beni Health Zone), which was sent to the morgue for swab collection, and a live alert that was invalidated after further investigation at the Kasitu PoC. In Haut-Uélé, the single live alert notified to the Magambe PoC (Tuluba Health Center, Isiro Health Zone), originating from Bolebole and presenting with fever, vomiting, and asthenia, was validated and referred to the Mayogo CT.

In Bas-Uélé, all six reported alerts were investigated and ruled out; two new Points of Contact (PoCs) were activated there (PK3 Ango and PK5 Dakwa) and equipped by the WHO with handwashing stations, infrared thermometers, and leg coverings. No alerts were reported in Ituri, Tshopo, or South Kivu.

Table 4. Status of key indicators for PoC/PoE by province, as of September 4, 2026

Indicators	Ituri	North Kivu	South Kivu	Tshopo	Haut-Uélé	Bas-Uélé		Overall
Report completeness	92%	100%	55.5%	83.3%	64.0%		ND	78.9%
% of functional strategic PoE/PoC	9.1%	N / A	N / A	N / A	N / A	N / A	N / A	9.1%
Number of people who have switched to PoE/PoC	137,005	90,809	8095	12,221	12,849	6037		267,016
% of travelers screened:	98.0% 97.9%	98.7% 99.0%	99.0% 99.0%	98.1%				
% of travelers who washed their hands	98.0% 97.9%	98.7% 99.0%	99.0%			ND		98.1%
% of travelers made aware	99.6% 97.9%	100% 100%	99.0%			ND		99.0%
Number of alerts notified	0	2	0	0	1	6		9
Number of live alerts validated (suspected cases)	0	0	0	0	1	0		1
Number of bodies recovered today	0	1	0	0	0	0		1
% of suspected cases transferred to CT/CTE	N / A	N / A	N / A	N / A	100%	N / A		100%
% of lifeless bodies swabbed/secured	N / A	100%	N / A	N / A	N / A	N / A		100%
% of suspected cases investigated within 24 hours	N / A	100%	N / A	N / A	100%	N / A		100%

1.2.2. Challenges

Operationalization of PoE/PoC is suboptimal: reporting completeness remains low in Tshopo (83.3%, 15/18) and Bas-Uélé has not reported. In Ituri, 1 (9.1%) of the 11 strategic PoE/PoCs remains operational without interruption, while the Avakubi PoC remains inoperative for security reasons. In Tshopo, 12 PoCs are activated out of 30 planned.

1.3. Laboratory

1.3.1. Main actions

- **Ituri : 53 new positive results** (37 living and 16 deaths) out of 341 new samples received and analyzed (**positivity: 15.5%**).
- **North Kivu : 31 new positive results** (17 living and 14 dead) out of 189 samples analyzed (**positivity of 16.4%**).
- **Haut-Uélé: 2 new positive results** (2 live) out of 37 samples analyzed (**positivity: 5.4%**).
- **Bas-Uélé: 5 samples** received and tested (3 live and 2 dead), **all returned negatives**.
- **Tshopo: 3 samples** received and tested for analysis (3 live), **all came back negative**.

1.4. Infection Prevention and Control (IPC/EDS)

1.4.1. Main actions

- **In Ituri**, 12 of the 41 planned infection control rings were opened (29.0%), 69 of the 71 identified households were decontaminated (97.2%), and 4 of the 5 health facilities were decontaminated (80.0%); no infection prevention and control (IPC) kits were distributed to households. Regarding the emergency data collection (EDS), 80 alerts were recorded, including 6 postponements from the previous day, 59 EDS were carried out, and 52 bodies were swabbed, with 7 cases being postponed to the following day.
- **In North Kivu**, the EDS pillar received 44 alerts in addition to 21 reports from the previous day, carried out 46 EDS and swabbed all 44 bodies (100%), without any failures; 16 cases are postponed to the following day.
- **In Tshopo**, no ring could be opened around the 20th, 21st and 22nd cases, due to community resistance for the first and the lack of address for the other two. Monitoring and support covered 11 ESSs with briefing of 112 providers and 51 PCI and SEHA supervisors, as well as the installation of 7 triage units; 46 of the 79 ESSs under surveillance (58%) have PCI kits and a triage circuit.
- **In Bas-Uélé**, briefing of 52 healthcare providers including 13 hygienists on hand hygiene, injection safety and waste management, provision of PCI/WASH inputs to 10 PoCs in 4 health zones (Buta, Koteli, Titule and Ganga) and PCI assessment of 4 ESS (CSR Amadi 72%, CSR Kembisa 71%, CS Baruti 39.5% and CS Agri Uélé 7.5%).

1.4.2. Challenges

- **Weak implementation of IPC standards:** in Ituri, the opening of rings remains very incomplete (29.0%, 12/41) and decontamination teams remain insufficient in Fataki, Mangala and Nizi; in Bas-Uélé, no rings opened nor households decontaminated, the waste area of the Buta General Referral Hospital is not functional and the Ganga and Viadana General Referral Hospitals remain without a triage circuit; In Tshopo, community resistance and lack of address block the opening of rings in the last three cases, IPC inputs remain insufficient in the ESS (only 1 ESS out of 79 with a score above 80%) and the incinerator of the Makiso-Kisangani General Referral Hospital remains out of order.

1.5. Holistic care

1.5.1. Main actions

- **In Ituri**, there were 96 new admissions to the various dedicated facilities, compared to 110 discharges, including 19 recoveries. Thus, 495 patients are hospitalized in 978 beds, representing an occupancy rate of 50.6%.
- **In North Kivu**, 78 new admissions and 58 discharges were recorded, including 2 recoveries; 237 patients are hospitalized expressing an overall bed occupancy rate in CTE/CT is in oversaturation (107.7%; 220 beds available).
- **In Haut Uélé**, there were 10 new admissions and 7 discharges (no recoveries). Thus, 62 patients are hospitalized in 128 beds, representing an occupancy rate of 48.4%.
- **In Tshopo**, no new admissions were recorded and one non-case was discharged. Seven (7) patients are in isolation for 25 beds, representing an occupancy rate of 28.0%.
- **In South Kivu**, there were 5 new admissions and no discharges. Sixteen (16) patients are in isolation out of 25 beds, representing an occupancy rate of 64.0%.

1.5.2. Challenges

Limited capacity and overcrowding: in North Kivu, some patients are treated in non-standardized treatment centers; in Ituri, the Ebola Treatment Center (ETC) at the Medical Center is at capacity for confirmed cases, as are the sites in Nizi, Lita, Mambasa, and Logo for suspected cases, with a delay in expanding services to Nizi and Bambu. In Tshopo, the unavailability of safe blood persists, and the technical infrastructure remains insufficient for biochemical and hematological tests; in Bas-Uélé, the lack of a standardized ETC/CT and ambulance services continues.

1.6. Vaccination

1.6.1. Main actions

Vaccination with the ERVEBO vaccine continued in Bas-Uélé, in the Buta health zone, for frontline workers and high-risk contacts, under the supervision of the teams (**cumulative number of people vaccinated: 486**) ; the list of people to be vaccinated continues in the Aketi health zone. In Tshopo, support for the supervision of vaccination teams for people living with HIV continued.

1.6.2. Challenges

Lack of mobility for vaccination teams in advanced strategy and absence of microplan for vaccination of PPL in 6 health zones of Bas-Uélé (Aketi, Ango, Bili, Buta, Ganga and Viadana);

1.7. Continuity of Care

In Bas-Uélé, support is being provided to laboratory and surveillance teams for the investigation. In Bas-Uélé, follow-up of 16 high-risk contacts under prophylaxis continues in the Buta and Ganga health zones, and three health centers (CSR Rubi Lisanga, CS Mobenge, and CS du Congo) are being assessed. In Tshopo, three people with HIV arrived from the Bafwasende health zone and began their preventive treatment.

1.8. Risk communication and community engagement

1.8.1. Main actions

- **In Ituri**, a meeting was organized by the Chief of the Group with the population of the BAYAKU Group, a group comprising 11 different villages. On this occasion, the village chiefs pledged to support 9 people, including 5 women, through individual interviews with family members, caregivers, and nurses at the CTE; 35 people, including 16 women, also benefited from awareness-raising activities.

- **In North Kivu**, 13,465 home visits were conducted, reaching 40,196 people; 49 educational talks were held with 1,638 people; 3 briefings (93 people), 5 community dialogues (108 people), and 6 mass communication campaigns (5,035 people) were organized; 5 out of 8 feedback and infodemics were resolved within 48 hours, and 156 alerts were reported by community stakeholders. A forum was held for 15 managers of private health facilities in Butembo to discuss contact tracing and alert reporting.
- **In Tshopo**, 1,302 people were sensitized (729 women and 573 men); 2 refusals were addressed out of 3 identified, 2 rumors were clarified, and 2 community feedback sessions were collected; 106 leaflets were distributed, and 2 articles were published by online media; 36 people received training in CREC in Belika (Bafwasende Health Zone), and students at the University of Kisangani were sensitized.

1.8.2. Challenges

Persistence of community resistance: in Ituri, 21 refusals of isolation at the CTE requiring reinforcement of the CREC and 7 bodies not swabbed due to community resistance; in North Kivu, the escape of a confirmed patient from the Katwa health zone and the refusal of a confirmed patient from the Lubero health zone to join the transit center, with the threat from the population to burn down the CTE under construction; in Tshopo, resistance blocking the opening of the ring for the 20th case and persistence of refusals of investigation, sampling and transfer to the CTE.

1.9. Mental Health and Psychosocial Support (MHPSS)

1.9.1. Main actions

- **In North Kivu**, 28 new confirmed cases and 34 of the 96 suspected cases benefited from SMSPS interventions; 50 test results were announced, including 28 positive results; 54 people receiving support were assisted; 93 households received support; and 14 recovered individuals received community follow-up. In Haut-Uélé, 24 confirmed cases received support, and 12 test results were announced, including 4 positive results.

1.9.2. Challenges

In Ituri, low coverage of SMSPS interventions around the latest confirmed cases in the health zones of Bunia, Lita and Komanda; in Tshopo, absence of SMSPS providers in Wanie-Rukula, Bafwasende and Isangi.

1.10. Logistics

1.10.1. Main actions

- **In Ituri**, organization of 4 inter-CTE transfers (CME, Rwankole and HGR Bunia) and support food in 462 of the 495 hospitalized patients (93%).
- **In North Kivu**, 7 calls were received, resulting in 7 successful dispatches (100%), with 4 ambulances mobilized out of 11 available (36.3%). In Tshopo, 4 vehicles were provided to 4 health zones in the city, and construction continued on the isolation sites in Kongakonga, Madula, and Kabondo, as well as the large-capacity Ebola Treatment Center (ETC).
- **In Bas-Uélé**, support for the mobility of health zone teams for the verification of alerts and packaging of IPC inputs and care intended for the Ganga health zone.

1.10.2. Security

1.10.3. Main actions

- **In North Kivu**, seven community policing activities were carried out: security escort for EDS teams for the burial of confirmed cases at the cemeteries of Bundji, Butala, Kasitu, and Kimbangu, and coverage of IPC teams in two households in the Butsili health district. In Bas-Uélé, a security assessment was conducted at the PK4 Kisangani road point of control, which is operational 24/7 without any major incidents.

1.10.4. Challenges

Twenty-one (21) refusals to self-isolate at the CTE were reported in Ituri. In North Kivu, officials resisted compliance with the health protocol at the PoCs, and the population threatened to burn down the CTE under construction in Katwa.

1.11. PSEA (Prevention of Sexual Exploitation and Abuse)

1.11.1. Main actions. • In Ituri, 107

agents (78 men and 29 women) were briefed on the prevention of sexual exploitation, abuse, and harassment and signed the code of conduct (Bunia and Mambasa Health Zones). In Bas-Uélé, 60 people were sensitized on the Prevention of Sexual Exploitation and Abuse (PSEA), bringing the total to 76, and "Zero Tolerance for Sexual Exploitation and Abuse" signs were installed at the Provincial Health Directorate (DPS), the WHO, and the Buta General Referral Hospital (HGR).

1.11.2. Challenges

- Still insufficient coverage of PSEA activities, with only 10 of the 36 health zones in Ituri (27.7%) aware of the reporting mechanism; lack of mapping of stakeholders; insufficient mass awareness materials; lack of transport and computer equipment for the pillar.

KEY MESSAGES

• Maintaining performance in terms of epidemiological investigations (**100% of suspected cases investigated**) and a stable daily proportion of contact tracing (**86.0%**)

which remains above the threshold of 85.0%.

• Continued notification in the provinces of Ituri, North Kivu and Haut-Uélé highlights sustained community transmission of the disease with a new health zone affected in North Kivu (**Kayna**).

• Thus, **86 new confirmed cases**, including **30 community deaths and 9 deaths within treatment centers**, were recorded in the last 24 hours, bringing the national total to **6,522 confirmed cases and 3,134 deaths (lethality: 48.1%)**.

• Ituri remains the epicenter of the epidemic, concentrating **80.9 %** of cumulative confirmed cases and **61.6%** of new cases reported in the last 24 hours, while North Kivu province holds **36.0%** of new cases with the highest lethality as well as **a new affected health zone**.

• Strengthening priority interventions (surveillance, care, IPC, logistics and community engagement) remains essential to quickly interrupt transmission chains.

TIMELINE OF KEY EVENTS

• From April 24 to May 15, 2026, the epidemic progressed from the detection of the first suspected cases to the biological confirmation of the Ebola Bundibugyo virus, leading to the official declaration of the 17th Ebola Virus Disease epidemic in the Democratic Republic of Congo.

• Between May 17 and 18, 2026, national authorities, with the support of partners, strengthened the response by declaring a national and then continental public health emergency, accompanied by mobilization

funds.



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